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Queensland Government

Residential Care Facility/ Multipurpose Health Service Post-Fall Clinical Pathway

(Affix identification label here)

URN:

Family name:

Given name(s):

Address:

Date of birth: Sex: M F I

- Clinical pathways **never replace clinical judgement**
- Care outlined in this clinical pathway **must be altered if it is not clinically appropriate** for the individual care recipient
- This pathway is to be used for any care recipient who has had a fall
- **V** indicates a variance from the pathway, document in clinical notes

Immediate Actions

- **Commence DRSABCD (Danger, Response, Send for help, Airway, Breathing, CPR, Defibrillate – if available) or as per local procedure**
- **Call for assistance**
- **Do not move the care recipient until assessed for injuries and safety**
- **Observe for symptoms of head and/or muscular skeletal injury (e.g. any change in behaviour, change in level of consciousness, headache or vomiting, any deterioration) – call 000 where required and/or immediately verbally contact GP/NP for advice**

Details of Fall and Initial Actions

Date: / /	Time found: :	Respiratory rate:	O_2 saturation: %	Blood pressure: /	Heart rate:	GCS score:	Temperature: °C	BGL:

Was the care recipient **unconscious**? ☐ Yes ☐ No		Obvious major skeletal deformities/fracture/injury? ☐ Yes ☐ No	
Major head trauma? ☐ Yes ☐ No		Did the care recipient show signs of increased confusion? ☐ Yes ☐ No	
Is the care recipient for hospital transfer? ☐ Yes ☐ No		Does the care recipient have a documented AHD? ☐ Yes ☐ No	

All care givers who initial are to sign signature log — 🗝 *Key* ■ Medical ▲ Nursing

Category	🗝		Initials	V
Medical assessment	▲	• Do any of the following apply to the care recipient: ☐ known coagulopathy ☐ on anticoagulant/antiplatelet therapy ☐ fall from greater than one (1) metre in height ☐ suspected head injury ☐ recent surgery/procedure ☐ other:		
	▲	• Verbally contact the GP/NP (ED/MO after hours) **Who was notified?** ..... **Date:** ... / ... / ... **Time:** ... : ...		
		• Consider pain management		
Investigations/ observations Document in observation chart at the following intervals	■	• **Suspected head injury or unwitnessed fall** What: neuro obs, respiratory rate, O_2 saturation, blood pressure, heart rate, BGL (as per local policy) When: Day 1: ½ hourly for 2 hours, *if normal →* hourly for 4 hours, *if normal →* 2nd hourly for 4 hours, *if normal →* 4th hourly for 14 hours, *if normal →* Day 2: 4th hourly for 24 hours ***OR*** observation as per medical order If GP/NP unavailable, contact ED/MO, RaSS, Frailty Intervention, RADAR team or Acute care for advice		
		• **No head injury** What: respiratory rate, O_2 saturation, blood pressure, heart rate, BGL (as per local policy) When: hourly for 4 hours, *if normal →* 2nd hourly for 6 hours, *if normal →* 4th hourly for 24 hours ***OR*** observation as per medical order		
		• **If there is a reduction in GCS score of ≥2 points or deterioration of observations (any change in behaviour, headache, vomiting or indications of internal bleed) call 000 immediately and verbally contact GP/NP/ED/MO immediately. Review Advance Care Planning documents (e.g. Statement of Choices, AHD, ARP, EPOA)**		
Management plan (within 24 hours) Note that there may be late manifestations of head injury or other injury after 24 hours	▲	• Notify family of incident as soon as possible (as agreed with family and as per local policy)		
		• Treatment plan as per GP/NP/ED/medical orders/discharge plan (e.g. palliation, End-of-Life Care Pathway, surgery, pain management)		
		• Document incident and outcome in care recipient's clinical record		
		• Log incident report		
		• Communicate incident, outcomes and planned care at handover/transfer of care		
		• Continue as per care recipient care plan and medical advice		
		• Review Falls Assessment and Management Plan		

Signature Log (Every person documenting in this pathway **must** supply sample of their initials in the signature log below)

Initials	Print name	Designation	Signature	Initials	Print name	Designation	Signature

DO NOT WRITE IN THIS BINDING MARGIN

RESIDENTIAL CARE FACILITY/MULTIPURPOSE HEALTH SERVICE POST-FALL CLINICAL PATHWAY

v5.00 - 09/2025
WINC Code: 1NY31880
SW137

Residential Care Facility Post-Fall Clinical Pathway

Immediate actions

- **Commence DRSABCD (Danger, Response, Send for help, Airway, Breathing, CPR, Defibrillate – if available) or as per local procedure**
- **Call for assistance**
- **Do not move the care recipient until assessed for injuries and safety**
- **Observe for symptoms of head and/or muscular skeletal injury (e.g. any change in behaviour, change in level of consciousness, headache or vomiting, any deterioration) – call 000 where required and/or immediately verbally contact GP/NP for advice**

Care Recipient Fall (witnessed or unwitnessed)

Initial assessment

- Document initial observations
  - » respiratory rate, O_2 saturation, blood pressure, heart rate, GCS, temperature, Blood Glucose Level (BGL)
- Document the following:
  - » consciousness
  - » major head trauma
  - » obvious major skeletal deformities/fracture/ injury
  - » signs of confusion
- Is the care recipient for hospital transfer?
- Does the care recipient have a documented AHD?

Medical assessment

- Verbally notify GP/NP/MO after hours to review if any of the following apply to the care recipient:
  - » known coagulopathy
  - » on anticoagulant/antiplatelet therapy
  - » fall from greater than one (1) metre height
  - » suspected head injury
  - » recent surgery/procedure
- Consider pain management
- Document who was notified and when
- If GP/NP unavailable contact Local RaSS/ED/Frailty Intervention or RADAR team
- If GP/NP/RaSS/Frailty or RADAR Team unavailable and vitals signs stable, baseline physical and neurological function remains intact, no obvious injuries and illness continue monitoring as per pathway. Notify GP/NP as soon as possible

Investigations/observations

No head injury

What:
- » respiratory rate, O_2 saturation, blood pressure, heart rate, BGL (as per local policy)

When:
- » hourly for 4 hours, *if normal* →
- » 2nd hourly for 6 hours, *if normal* →
- » 4th hourly for 24 hours;
- » ***OR*** observations as per medical order

Suspected head injury or unwitnessed fall

What:
- » neuro obs, respiratory rate, O_2 saturation, blood pressure, heart rate, BGL (as per local policy)

When – Day 1:
- » ½ hourly for 2 hours, *if normal* →
- » hourly for 4 hours, *if normal* →
- » 2nd hourly for 4 hours, *if normal* →
- » 4th hourly for 14 hours;
- » ***OR*** observations as per medical order

When – Day 2:
- » 4th hourly for 24 hours;
- » ***OR*** observations as per medical order

If there is a reduction in GCS score of ≥2 points or deterioration of observations (any change in behaviour, headache, vomiting or indications of internal bleed) call 000 immediately and verbally contact GP/NP/ED/MO immediately. Review Advance Care Planning documents (e.g. Statement of Choices, AHD, ARP, EPOA)

Management plan (within 24 hours)

Note that there may be late manifestations of head injury after 24 hours

- Notify family of incident (as agreed with family and as per local policy)
- Surgical intervention/treatment plan as per GP/NP/ED/ medical orders/discharge plan (e.g. palliation, End-of-Life Care Pathway, surgery, pain management)
- Document incident and outcomes in care recipient's clinical record
- Log incident report
- Communicate incident, outcomes and planned care at handover/transfer of care
- Review Falls Assessment and Management Plan
- Continue care as per care plan and medical advice
- Review falls prevention strategies

Legend									
AHD	Advance Health Directive	**ED**	Emergency Department	**GCS**	Glasgow Coma Scale	**MO**	Medical Officer	**RADAR**	Residential Aged Care District Assessment and Referral Service
ARP	Acute Resuscitation Plan	**EPOA**	Enduring Power of Attorney	**GP**	General Practitioner	**NP**	Nurse Practitioner	**RaSS**	Residential Aged Care Facility Acute Support Service

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